

Introduction to Health Education

Jote M.C

Definition and concepts of health...

Health is:

“A state of complete:

- Physical**
- Mental**
- Social well-being not merely the absence of disease or infirmity.”**

WHO

❖ This classic definition is important, as it identifies the vital components of health.

HEALTH EDUCATION

- Health education is a **part of health care** that is concerned with **promoting healthy behaviors**
- A person's behavior may be the main causes of health **problems**, but it can also be the **main solution**.
- It is through health education we help people to understand their behaviors and how it affect their health and encourage them to make their own choices
- It is voluntary education

HEALTH EDUCATION...

In health education we encourage behaviors that:

- ❖ Promote health as a part of promotive health care
- ❖ Promote behaviors that prevent illness or diseases as a part of preventive health care
- ❖ Promote behaviors that facilitate cure diseases as apart of curative health care, and
- ❖ Promote behaviors that facilitate rehabilitation as apart of rehabilitation health care.

HEALTH EDUCATION...

- **Health Education** is “Any combination of learning experiences designed to facilitate voluntary action conducive to health”

Elaboration of the definition:-

- ❖ **Combination**: it emphasizes on the importance of matching multiple determinants of behaviors with multiple learning experiences or educational intervention

HEALTH EDUCATION

- **Designed:** distinguishes health education from incidental learning experiences as a systematically planned and organized activity
- **Facilitate:** creating favorable condition such as predispose, enable, reinforce

HEALTH EDUCATION

Voluntary: with full understanding and acceptance of the purpose of the action.

❖ Without use of coercion or any manipulative approaches.

❖ In health education we do not force the people to do what we want them to do, instead our effort is to help people to make decisions and choices by themselves.

Action: behavioral steps/measures taken by individuals, groups or community to achieve the desired health effect.

HEALTH PROMOTION

- ❖ To reach a state of complete physical, mental and social well being it is beyond the activity of health education and even health sectors.
- ❖ In other words health is not just the responsibility of health sector alone. But the responsibility of every sectors working for development.
- ❖ Thus, the concern for health outside to the health care sector is the call for health promotion

HEALTH PROMOTION

❖ “A combination of educational and environmental supports for actions and condition of living conducive to health.” *GREEN AND KRUETR (1991)*

Combination: refers to the necessity of matching multiple determinants of health with multiple intervention or sources of supports.

HEALTH PROMOTION

Educational: refers to the communication part of health promotion. That is health education.

Environmental: refers to the social, political, and economic, organizational, policy and regulatory circumstances influence behavior or more directly health

HEALTH PROMOTION...

Elements/component of health promotion

A. Health education and

B. Environment

- ❖ Political environment
- ❖ Social environment
- ❖ Economic environment
- ❖ Organizational environment

APPROACHES IN HEALTH PROMOTION

Health promotion employs two approaches:

A. Educational approach (health education):-

- Attempt to influence predisposing factors through **direct communication**, reinforcing factors through indirect communication in social environments to create norms and values that support life style conducive to health and enabling factor through trainings and organization.

APPROACHES HEALTH PROMOTION...

B. The ecological /environmental approach (political action)

- ❖ It employs policy, organization, and regulation to influence the enabling and reinforcing factors for environmental and life style changes supportive of health.

eg,. Control of cigarette, alcohol

HEALTH INFORMATION

- It is health facts disseminated to the target audience focusing on the basic facts related to the health issue under consideration.

HEALTH INFORMATION...

- ❖ Health information is the scientific facts (e.g. the causes, mode of transmission, sign and symptoms ,prevention methods of particular diseases) content of health education which primarily aimed at increasing of knowledge on that particular health problems;
- ❖ but health education address the other factors that affect health behaviors other than knowledge such as beliefs, attitude ,reinforcing factors etc.

HEALTH INFORMATION...

- ❖ **Health communication** is the process by which two or more people exchange ideas, facts, feelings or impressions about health in ways that each gains a **common or mutual understanding** of the meaning and the use of the message related to health under consideration.

Information, Education and Communication (IEC)

IEC is a process of working with individuals, communities and societies to:

- develop communication strategies to promote positive behaviors which are appropriate to their settings.

Behavior Change Communication

- **BCC** is a Process of changing social and individual **attitude** and **behavior** by providing them with relevant information, education and motivation through appropriate channel.
- **BCC** is a programme or activity that is specifically designed to change or sustain the behaviour of individuals or social groups by using variety of communication techniques.

Rationale for health education

- A. The continued existence and spread of *communicable* diseases such as malaria, TB, HIV/AIDS that need the involvement of the community members and environmental interventions.
- ❖ Nearly three million people die each year from TB, most of which are in developing world.
 - ❖ About 75% of *childhood illnesses* are preventable e.g. measles by immunization, malnutrition and diarrhea by teaching mothers about good weaning foods and promoting breast feeding up until age of two.

Rationale *for health education*

B. For some diseases health education is the only practical choice in order to prevent the spread of the disease or to lead a normal life.

E.g. HIV/AIDS.

C. The tendency of increasing magnitude of *chronic conditions* and other *emerging agendas*, which required the cooperation of individuals to lead with the problem. E.g. Hypertension.

Rationale *for health education*

- D. Human behaviors are almost the single causes for the development of such currently emerging health problems and also the main solution.
- E. Many people are ignorant of the causes of their illnesses and may be afraid of seeking treatment and hide their illness not to be considered, as an “*outcast*” from society and many others do not seek treatment until it is too late.

Rationale for health education

- F. Prevention is better than cure, is a statement which is generally true especially for non communicable diseases: it is much cheaper, prevents economic loss of manpower hours, and lessens worry from the money spent on health bills in the family.*
- G. Increasing threats to the young from new and harmful behaviors.
E.g.. tobacco use, teen-age pregnancy, substance use, etc.
- H. Increased awareness of people on chronic health problems and the need to know preventive actions.

PHC concepts and health education

- Health education was identified as a primary means of "Health for All."
- Its central role is in improving community participation and building the capacity of communities to make decisions.
- Health education should aim at enabling people to make choices.

Importance of health education in PHC

- 1) Promoting community involvement and self-reliance
- 2) Enhancing decision-making skills at the local level
- 3) Allowing for a diversity of objectives in formulating policy
- (4) Harmonizing national and local plans
- (5) Facilitating Inter-sectoral action
- (6) Using appropriate technology
- 7) Measuring Community involvement and the impact of health education.

The health policy of Ethiopia

- “IEC of health shall be given appropriate prominence to enhance health awareness and to propagate the important concepts and practices of **self-responsibility** in health”
- **Health Education** shall be strengthened generally and for specific target populations through the mass media, community leaders, religious and cultural leaders, professional associations, schools and other social organizations for...”

The health policy of Ethiopia

- “We must recognize that most of the world’s major Health problems and premature death are preventable through **changes in human behaviors** and at **low cost**.”
- We have the know –how and technology but they have to be transformed into effective action at the community level”

Goals of health education

1. Health consciousness :

- ✓ raise health consciousness or awareness of health issues.

2. Knowledge :

- ✓ give specific knowledge and information.

3. Self awareness :

- ✓ Concerned with exploring feelings and being aware of one's feeling in relation to particular health issue or to health in general.

Goals of health education...

4. Attitude change :

- ✓ Concerned with changing what people feel, what they believe and what their opinion is.

5. Decision making :

- ✓ Involves both "knowing" and "feeling" objectives and is concerned with deciding what to do in the future about health or a particular aspect of health.

Goals of health education...

6. Behavior change :

- ✓ concerned with carrying out a decision and actually doing something about a health matter.

7. Social change :

- ✓ changing the physical and or social environment so that people are encouraged to adopt healthier behavior

Goals of health education

Prevent premature deaths that are related to
lifestyles and behaviour

Promote, maintain, and improve individuals and
community health

Positive, informed changes in health behavior

To improve quality of life

Aims of HE

- To help each individual, each family to exercise their right to achieve the harmonious development of their physical, mental and social potential

Scope of HE

- During 1970s,1980s –individual's behavior has given emphasis as determinant of health status
- This has eclipsed the attention to the broader social determinants of health.

Scope...

- Interventions on social and behavioral factors related to health should link multiple levels of influence, including the individual, interpersonal, institutional ,community and policy levels
- Community and societal influence on health behavior and strategies to effect community and social policy changes in addition to individual level.

Scope..

❖ Behavior is viewed as being affected by and affecting ,multiple levels of influence for health related behaviors and conditions

- ✓ Intrapersonal/individual factors
- ✓ Interpersonal factors
- ✓ Institutional/organizational factors
- ✓ Community factors
- ✓ Public policy factors

Principles of Health Education

1. Definite aim

- have a well defined objective.
- Know what you want to accomplish
- Plan your health education program
- Specify target audience
- Segment audience and tailor message

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2. Credibility

- should be based on facts
- it must be consistent and compatible with the scientific knowledge and also with the local culture and educational system

3. Interest

- It is known that people are unlikely to listen to things that are not of their interest or concern
- find out the real health needs of people.

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4. Participation

- is based on the principle of active learning.
- A high degree of participation tends to create a sense of involvement, personal acceptance, and decision making

5. Motivation

- Motivation is said to be contagious; one motivated person can spread motivation throughout a group, if not throughout a given community.

6. Comprehension

- know the level of understanding, educational status, and literacy of the people to whom the teaching is directed

7. Re-enforcement

- repetition of the health education program is necessary.
- If a program is not re-enforced, the understanding of people possibly returns to its pre-awareness stage.

8. Learning by doing

- Learning will be best when supported by practical sessions such as demonstrations and field attachments instead of mere explanation of facts.

9. From known to unknown

- start from what people know (simple) Proceeding to what people don't know (complex) to ensure their participation and confidence

10. Role model (setting examples)

- health educators, themselves, should be model to what they educate. (do not smoke)

11. Good relations

- Good relationship (rapport) with audience should be established by sharing information, ideas, and feelings transparently so that there could be a process of establishing and maintaining trust.

12. Leadership

- people learn best from individuals whom they respect and appreciate, and hence it is good to penetrate through leaders (gate keepers) to the community in health education intervention.

13. Feed back

- For effective communication process, response of the audience is very important to confirm that the learning objective is already achieved or not.

14. Successive Experience

- People tend to adopt those practices that give them satisfaction and reject that end with unhappy experiences and hence it is necessary to provide satisfying experience in order to establish a new practice.

15. Group support

- Any new idea or practice will be feared at first; it is only when many people support the new idea that the individual members come out for adoption

Challenges to health education

- Changing Health behavior is conditioned by factors such as social, psychologically, economically ,quality services, which are difficult to deal with simultaneously
- People are preoccupied with many daily activities to support their life.

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- Health education is not considered important during normal life;
People are concerned about diseases
- People engaged with H/E programs lack special training
- Methodological challenges,theoretical& substantive challenges

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- have limited specialized training and therefore have, In some cases, restricted their activities
- Many health educators do not have access to the tools required to be effective practitioners

Fayyaa ta'aa!